

Review the adjudication portal - 09/15/2026

Attendees: TeamsMaestro - AI notetaker, Tariro Makadzange, Tinotenda Chibongore

Overview

Tariro and Tinotenda reviewed the EOPE/LOPE adjudication portal in demo mode to validate roles, data flows, and user experience. They aligned on simplifying the adjudicator workflow, tightening access controls for a regulated study, and increasing automation/AI use ahead of near-term launch milestones.

Key Takeaways

- Owner role to be created for Tariro with full visibility; IT remains Admin focused on user/integration management.
- User naming to be standardized (firstname lastname) and portal roles must align with the study Delegation Log.
- Medical Officer (data entry only; no adjudication visibility) and Medical Monitor roles to be added; sponsor medical monitor read-only; project manager access removed.
- "Manage Contract" to be removed from adjudicator profiles to avoid duplicating contracting processes.
- QC to shift from human checks to algorithmic validation; adjudicators will only see clean data.
- Monitor portal to be trimmed to core functions (Patient Database, Assignments); per-study lab reference ranges (EOPE/LOPE) to be embedded in code.
- Auto-assignment of adjudicators (A/B) from a pool with workload limits (e.g., ~50 cases each) to replace manual assignment.
- Adjudicator portal to be reconstructed: FDA-style medical narrative prominently at top; full labs organized by clinical panels with ranges and explicit "unavailable" flags; inclusion/exclusion checklist; ultrasound linkage; longitudinal summary retained and post-delivery visits color-coded; per-visit classifications prompted only after overall decision; confusing/biasing fields (site clinician opinion, derived phenotype, GA at event, DV26, redundant eSource) to be removed; required fields (e.g., differential diagnosis) enforced.
- References/work instructions to be embedded (ACOG/WHO/country guidelines, protocol, simple navigation guide) for fifth-grade-level clarity.
- Integration path to favor Oracle EDC if feasible; interim RealTime CSV ingest is complete with potential scripting to automate download/upload.
- Parallel AI strategy endorsed: AI-generated narratives and an AI adjudicator run alongside humans for comparison and potential publication.

Next Steps

- **Tinotenda:** Build an Owner (super user) portal/profile for full system visibility and adjust Admin scope.
- **Tinotenda:** Standardize user naming; clean test users; align each user's role with the Delegation Log.
- **Tinotenda:** Add Medical Officer (data entry only) and Medical Monitor roles; set sponsor monitor to read-only; remove project manager access.
- **Tinotenda:** Remove "Manage Contract" from adjudicator profiles.
- **Tinotenda:** Implement algorithmic QC for data ingestion and ensure adjudicators only receive clean datasets.
- **Tinotenda:** Reduce Monitor portal to Patient Database and Assignments; embed EOPE/LOPE lab reference ranges in code.
- **Tinotenda:** Implement auto-assignment (A/B) with capacity limits (e.g., ~50 cases per adjudicator).
- **Tinotenda:** Rebuild adjudicator portal around an FDA-style narrative (include inclusion/exclusion, clinical course, delivery summary), reorganize labs by panels with ranges and "unavailable" indicators, add ultrasound links, retain/improve longitudinal summary with post-delivery color-coding, simplify decision flow (overall first, then per-visit as needed), remove confusing fields, and enforce required fields including differential diagnosis.
- **Tinotenda:** Add in-portal references (protocol, ACOG/WHO/country guidelines) and a simple work instruction for navigation.
- **Tinotenda:** Confirm integration approach (Oracle EDC API vs RealTime); if delayed, script CSV download/upload automation with Elliot.
- **Herald:** Provide EOPE/LOPE lab reference ranges from the lab operations manual for embedding.
- **Emmanuel:** Advise on feasibility/timeline for Oracle EDC vs RealTime API integration.
- **Elliot:** Automate current CSV data pipeline pending API integration.
- **Tariro:** Share/approve the FDA-style narrative template and inclusion/exclusion checklist; validate prototypes upon receipt.
- **Tinotenda:** Deliver updated prototypes and links for review and follow up for feedback.

Key Topics

Portal status and access method [Update](#)

- Tinotenda confirmed the adjudication portal was still in demo mode and not yet on ERP; hosting on ERP would occur after build validation.
- Login used Microsoft SSO with an initial password creation for security.
- Tariro began the session by attempting to access the review adjudication portal via a shared login link pasted in the meeting chat.

Admin portal overview and user management [Update](#)

- Tinotenda described the admin dashboard as an overview of studies, sites, users, audit trails of logins, and system activity.

- User management allowed adding users, assigning roles via a dropdown (e.g., admin, monitor, chairperson), creating temporary passwords, and changing roles.
- Two monitor roles existed: QC reviewer and RA reviewer.
- Tinotenda proposed the admin be an IT person to delegate roles and permissions; Tariro requested she also be an admin for full visibility.

Standardizing user naming and aligning roles with delegation log Decision

- Tariro requested standardized user name formats (e.g., "Tawanda Cuisina" without titles) across accounts.
- Tariro stated all users and their roles must align with the study delegation log, and test/dummy users (e.g., Alton, Panache) should be cleaned up.
- Tariro noted specific individuals should or should not have access: Cuisinha should not be an adjudicator; Gamachira was a sponsor-side medical monitor and could likely have read-only view of adjudication results; Kuda and Melissa (medical monitor) could have access; project managers should not be on the portal.

Role definitions and missing roles (Medical Officer, Medical Monitor) Decision

- Tariro noted missing roles in roles/permissions: "medical officer" and "medical monitor".
- Requirements set: Medical Officer could input data and address queries but must not see adjudication results; Medical Monitor could see cases and adjudication results (sponsor monitor view-only, no edits).
- Tinotenda explained medical monitors had their own monitors portal split into two roles (data ingestion and assigning/reviewing), with a potential super user who could do both; a custom role draft feature existed for adding roles.
- Tariro insisted she needed full system visibility at all times, beyond admin's current scope.

Owner vs Admin scope; need for an Owner (super user) portal Decision

- Tinotenda said the Admin portal was scoped to user management, site configuration, mapping canonical fields, and app integrations (Outlook, SharePoint, Teams) per SOPs; Admin could not view blinded adjudicated cases.
- Tariro requested an "owner" role with full visibility across the entire system in addition to admin; she and IT should be admins, and she should have full access control.
- Tinotenda agreed to prototype an Owner's portal with super user capabilities.

Remove 'Manage contract' from adjudicator profile Decision

- Tinotenda showed an adjudicator profile action labeled "Manage contract" intended to track contract start date and study assignment (EOPE vs LOPE) and to support billing per case adjudicated at month-end.
- Tariro asked to remove this from the portal to avoid duplicating existing contracting processes; billing would be based on number of cases adjudicated.
- Tinotenda agreed to remove or replace this action.

Committee assignments and chairperson activation Update

- Tinotenda explained a chairperson portal for non-concordant cases allowed setting meetings and arbitration notes.
- Activation in Committee Assignments ensured only one chairperson was active at a time; current accounts were dummies for testing.
- Tariro parked deeper review for later, asking first to finish access, people, roles, and permissions.

Portal permissions matrix and visibility constraints Decision

- Tinotenda stated the admin portal managed clinical operations, governance, and access review; it was denied access to blinded case content.
- Tariro reiterated the need to add explicit roles for Medical Officer and Medical Monitor with appropriate visibility limits and to ensure regulated activity constraints and blinding were respected.

Data sources, ingestion, and mapping (RealTime vs Oracle EDC) Update

- For testing, data came from RealTime CSV downloads hosted on secure servers; small data packets were used.
- Tinotenda planned to map/connect to RealTime upon validation; optionally also connect to Crelio for QC.
- Tariro asked how completeness was confirmed since EDC marked completion; Tinotenda showed a "RealTime imports" tab with pass/fail and field mapping checks.
- Tinotenda suggested Oracle EDC might be easier to integrate because the portal was hosted on Oracle Cloud; he cited Emmanuel's advice; if integration lagged, CSV downloads from EDC could be used with updated mapping scripts.
- Tariro asked if RealTime mapping was complete for a Friday launch; Tinotenda said mapping was complete for download-then-upload, and with Elliot's help the process could be automated.

QC approach: move from human checks to algorithmic validation Decision

- Tinotenda demonstrated ingestion outcomes including failed imports and duplications from RealTime's structure; monitors currently checked sources and row mappings.
- Tariro questioned human value in QC and requested replacing human QC with algorithms (or simple scripts) to validate, clean, and approve packages automatically.
- Tinotenda agreed that a script could handle these checks and would integrate algorithmic QC to minimize human input.

Monitor portal scope reduction and automation Decision

- Tinotenda showed the monitors portal: operational progress dashboard (studies, sites, adjudicators, adjudication status, visit numbers, date filters), imports, patient database, and assignments (including auto-assign A/B to all).
- Tariro advocated maximizing automation, including algorithmic QC and auto-assignment from the adjudicator pool; suggested capacity caps (Tinotenda mentioned 50 cases per adjudicator at a time).
- Lab reference ranges: currently defaulted to EOPE; Tariro required dynamic per-study ranges (EOPE vs LOPE) sourced from the lab operations manual (via Herald); ranges should be embedded in code, not

managed via UI.

- Tinotenda committed to scraping off unnecessary tabs (e.g., lab reference ranges), embedding static ranges, leaving primarily Patient Database and Assignments, and implementing auto-QC.

Adjudicator portal: need for FDA-style narrative and simplified clinician-centric UX Decision

- Tariro required a prominent adjudication narrative at the top (bright red/orange), written in FDA submission style: patient demographics and obstetric history, inclusion/exclusion criteria met, clinical course by visits (including delivery summary and outcomes), followed by the adjudication question (e.g., preeclampsia yes/no).
- She requested the narrative be auto-generated (AI or algorithm) and available without an extra button press; adjudicators should read it in ~10 minutes and then provide their opinion.
- She asked to include a references/work-instruction section with links to the protocol, ACOG, WHO, and country-specific guidelines; keep instructions at a "fifth-grade" clarity level to reduce support emails.

Adjudicator portal: data presentation and content requirements Decision

- Organize labs by clinical panels with reference ranges: hematology (e.g., hemoglobin, hematocrit, platelets), renal function (e.g., creatinine, BUN if present), urinalysis (e.g., protein levels, urine protein/creatinine ratio), LFTs, LDH, and other available parameters.
- Explicitly mark data as unavailable where missing to prevent misinterpretation as protocol deviation.
- Include ultrasound scan linkage (especially baseline dating) and any imaging; explain visit 5 as a delivery visit and show source (hospital medical records) with delivery outcome summary.
- Maintain longitudinal per-visit evidence visualization; color-code post-delivery visits differently to aid context.
- Simplify per-visit adjudication: capture overall adjudication first, then supply per-visit yes/no/classification for onset mapping only as needed.

Remove or clarify confusing/biasing elements in adjudicator view Decision

- Remove site clinician's diagnosis/opinion from adjudicator view to prevent bias.
- Remove derived phenotype, GA at event, DV26 score, and "available" flags that did not aid clinical decision-making.
- Clarify labels like "cumulative interpretation through this visit by site clinician" if retained; explain meanings inline to avoid late-night queries.
- Ensure required fields (e.g., differential diagnosis/alternative explanation) were mandatory before sign-and-lock; add inclusion/exclusion criteria checklist for quick review.

Chairperson portal review deferred

- Due to time constraints, the chairperson portal walkthrough was deferred.
- Tinotenda had planned to show it after monitors/adjudicators; Tariro prioritized adjudicator UX and core changes first.

Parallel AI adjudication and research opportunity Idea

- Tariro proposed a background AI adjudicator running concurrently with human adjudication using ACOG/WHO and country guidelines plus literature as knowledge base.
- Plan would enable comparison across ~1,000 EOPE and ~1,000 LOPE cases (example volumes mentioned qualitatively as "about a thousand or so" per study) to assess AI vs human adjudication; potential for a manuscript at the end.
- Random quality checks by medical monitors would validate AI-generated narratives and outputs.

Timelines and deliverables [Update](#)

- Tariro referenced a Friday launch; she asked if RealTime mapping was complete for this.
- Tinotenda committed to provide an update on Friday after scraping off nonessential elements, simplifying adjudicators' experience, and creating an owner role prototype.
- Tinotenda estimated a full revamp and deployment would take four days, targeting next week Monday, with interim prototypes for validation on Friday.
- Tariro asked Tinotenda to send links and actively follow up for feedback to "get this right."

Transcript

Tariro Makadzange: Hi, Tina Tender, how are you?

Tinotenda Chibongore: Hi Doc, how are you doing?

Tariro Makadzange: I'm fine, thank you. Sorry for jumping late. I'm just finishing off a previous call. Let me also just make sure I put this on my bracker. Are you here or are you dining in? Are you at the office?

Tinotenda Chibongore: I'm not at the office, but I live nearby, so I don't know if you. If

Tariro Makadzange: That's

Tinotenda Chibongore: you wanted

Tariro Makadzange: okay,

Tinotenda Chibongore: me to be. Yep.

Tariro Makadzange: you don't need to come through. Let me just. Let me do this. This is EOPE and lope, isn't it?

Tinotenda Chibongore: Yes.

Tariro Makadzange: Okay, good. So where is the portal on the erp? How do I access the portal and

Tinotenda Chibongore: Okay.

Tariro Makadzange: let me just get onto this review adjudication portal.

Tinotenda Chibongore: All right. So we are still in demo mode. We have not put it on ERP as it is still in development. So after you have validated the build, we are going to host it on erp. If you check this meetings chart, I am now pasting the link for the login. Yeah. So you sign in with Microsoft as always. Okay, so create a new password for security. All right, so this is the first portal. This is the admin portal. So as

Tariro Makadzange: Okay.

Tinotenda Chibongore: for. Yeah, so let me just. So many. So we have the dashboard. So the dashboard just gives you an overview of the whole thing. The, the studies we are doing, the sites, the users and so on and so forth. So it, it also gives an audit trail of logins and so on. So the dashboard is just an overview of the study and then going on to the users. That's where the main admin control is. So that's why there was a proposal that the admin should be, should be an IT person so that they can delegate the roles as well as the permissions. So you can change a person's role. You can also add users if you go on to add user right up there. Because we had a problem with people trying to log in with their CRMG accounts. So you can add them as an email and create a password for them, a temporary password that they will change as soon as they. They log in. So as you can see, where it says adjudicator, there's a Dropbox there. You can see all the kinds of roles you can assign. Just follow one user all the way to the end. You see that Dropbox? So you can change from admin to monitor to chairperson. As for the monitor, we have two different monitors. We have the QC reviewer and we have also the RA reviewer. So there are two different roles for the monitor as well. So also when you go. So is there anything you'd like to change on the user account management as we go on?

Tariro Makadzange: No, except one thing I would say is that we should standardize the naming

Tinotenda Chibongore: Okay.

Tariro Makadzange: so you know, we shouldn't have Dr. Cuzinha Elton, blah blah, blah. Just put.

Tinotenda Chibongore: Okay.

Tariro Makadzange: Put their name. So Tawanda Cuisina. So standardize the format. That's super, super important.

Tinotenda Chibongore: Okay.

Tariro Makadzange: Obviously

Tinotenda Chibongore: So.

Tariro Makadzange: Cuisinha would not be an adjudicator. The adjudicators

Tinotenda Chibongore: Yeah.

Tariro Makadzange: are few. So

Tinotenda Chibongore: Okay.

Tariro Makadzange: you said the monitor. They two monitor roles.

Tinotenda Chibongore: Yes.

Tariro Makadzange: Can you explain what they see?

Tinotenda Chibongore: Don't worry. We are going through all platals. I'm going to assign you as a monitor very soon so that you can. I can

Tariro Makadzange: No, no,

Tinotenda Chibongore: explain

Tariro Makadzange: no.

Tinotenda Chibongore: it

Tariro Makadzange: So

Tinotenda Chibongore: wisely.

Tariro Makadzange: I should be assigned as admin. So I agree with you with it

Tinotenda Chibongore: Yes.

Tariro Makadzange: being admin, but

Tinotenda Chibongore: Yes.

Tariro Makadzange: I need to have full visibility of the entire system at all times.

Tinotenda Chibongore: Okay.

Tariro Makadzange: So it should be myself and it as the admin

Tinotenda Chibongore: Okay.

Tariro Makadzange: and I should have full access control. Let's. Yeah, maybe we'll go to that stage. But we should really think about the users and the user access in

Tinotenda Chibongore: Okay.

Tariro Makadzange: part because this is a regulated activity. So

Tinotenda Chibongore: Yes.

Tariro Makadzange: you know, Kazinha should probably not be seeing. He might not even. Shouldn't probably even be on this portal. Or you have a

Tinotenda Chibongore: Okay.

Tariro Makadzange: medical. So you might have a medical officer role that can input

Tinotenda Chibongore: Okay.

Tariro Makadzange: data or address, you know, queries, but they should not be able to see the adjudication results, et cetera.

Tinotenda Chibongore: Okay.

Tariro Makadzange: That sort of interferes with the study

Tinotenda Chibongore: That's okay.

Tariro Makadzange: dynamics and study

Tinotenda Chibongore: Okay.

Tariro Makadzange: integrity. So their only role is to input data. So they might be very similar to the quality person who checks is all the information there. All the fields are there. And if things are missing, sort of asks the doctors, you know, I think these you are probably just setting up as dummy. But you know, Alton should

Tinotenda Chibongore: Yes.

Tariro Makadzange: not be and the manual should not be. Gamachira is a medical monitor on the spot sponsor side. So

Tinotenda Chibongore: Okay.

Tariro Makadzange: you should have a medical monitor of you. And I think since she is the sponsor, she probably can see what the adjudication results are. But maybe it's a view and then obviously nobody should be able to edit. Kuda can have access. Melissa as the medical monitor. The project

Tinotenda Chibongore: Yes.

Tariro Makadzange: manager should not really be on this. So I'm not sure. I don't think the project manager needs to be there. And similarly. Panache. I know this was just the setup

Tinotenda Chibongore: Yes.

Tariro Makadzange: stage.

Tinotenda Chibongore: This was just for testing.

Tariro Makadzange: Yeah.

Tinotenda Chibongore: Yes.

Tariro Makadzange: So,

Tinotenda Chibongore: Then

Tariro Makadzange: yeah, so then you'll need to clean it up. And then everybody,

Tinotenda Chibongore: okay.

Tariro Makadzange: their role must be on the delegation. Delegation log for the study. And this should align with the role in the delegation log.

Tinotenda Chibongore: All right.

Tariro Makadzange: So adjudicator profile.

Tinotenda Chibongore: All right. So for the profiles.

Tariro Makadzange: So when you correct that, when you correct the names, you'll put their information. What does it say? Manage contract.

Tinotenda Chibongore: Yes.

Tariro Makadzange: Is it

Tinotenda Chibongore: So

Tariro Makadzange: contact or contract? I can't read

Tinotenda Chibongore: it's,

Tariro Makadzange: this.

Tinotenda Chibongore: it's, it's contract. Contract.

Tariro Makadzange: So what is intended here?

Tinotenda Chibongore: So I, I heard that the adjudicators would be on a contract based model, so you would be able to add their contract start date, the study that they are active in. Because when they're adjudicating, we should be able to sort of divide the data between EOPE and Lope. So you are assigning them to either of the two between Lope and eoep. So you're now assigning them their contract study as well as the contract start date in their contract details. So an adjudicator cannot.

Tariro Makadzange: Why would we put that in this? Also, why do they need. Why does that need to be there? I'm trying to distinguish the value. Because they're going to adjudicate a case and in the case it says whether it's EOP or LOP

Tinotenda Chibongore: Yes.

Tariro Makadzange: at

Tinotenda Chibongore: Huh?

Tariro Makadzange: the end of the month. I don't know if they're being paid by the cases that they're adjudicating. But. But at

Tinotenda Chibongore: Okay.

Tariro Makadzange: the end of the month you just auto create an invoice based on the cases

Tinotenda Chibongore: Okay.

Tariro Makadzange: that adjudicated. Correct.

Tinotenda Chibongore: Yes. There's a billing invoice that comes from the number of cases that they would have adjudicated at the end of the month.

Tariro Makadzange: Okay, so I don't think this is necessary

Tinotenda Chibongore: Okay, so

Tariro Makadzange: unless you want them to sign their contract in here. But that creates a dual process with the existing contracting system. So personally I would not put that

Tinotenda Chibongore: all right.

Tariro Makadzange: there. You might have other actions, but certainly not manage contract.

Tinotenda Chibongore: Okay. All right. So going on to the committee assignments. So here we added a chairperson portal for the non concordant cases so that the chairperson can set meetings as well as arbitrate for the non concordant cases and then come back with the meeting notes. So when a chairperson is assigned on the user's role, you activate their account in the committee assignments so that we don't have a case where we have two or more chairpersons being activated at the same time. So as you can see here, these are all just dummy accounts for testing. So once I assign a chairperson, or you, since you are the admin, you assign a chairperson, you activate their account in the committee in the committee assignments. Yeah. So I'll take you through what the chairperson portal is. This is just for activation of the account.

Tariro Makadzange: Okay, all right, so maybe let's park that. So you'll show me the chairperson portal.

Tinotenda Chibongore: Yes.

Tariro Makadzange: Let's just finish this. Access and people first,

Tinotenda Chibongore: Okay.

Tariro Makadzange: then roles and permissions.

Tinotenda Chibongore: Yes.

Tariro Makadzange: Technical administrator. Okay, good. Clinical operations, study and content. No, Adjudicate, study and content. Okay. QA auditor, Independent review, Audit report. Okay. Governance reviewer, Law studies. Okay. Access reviewer. Okay, this sounds right. Portal permissions, matrix, blinded case. So the roles and permissions. It looks like you're missing though this administrative. Where does the medical monitor, the medical officer, the. Where are those sitting?

Tinotenda Chibongore: Okay. So for the medical monitors, they are sitting in their own portal called the monitors portal. It's divided into two. One can pull out the data, upload the data. The other one is just for assigning and reviewing the data. So we have for quality control, we have one who cleans out the data and one who ingests the data. Or they can be both. There's a super user that can be both for the sake of time. As for the roles and permissions, this one is for you to draft. Probably there is a role that's missing

that you have seen that is missing from the portal. You want someone who does a role that I've probably missed when building the portal. So there's a draft custom row up there which will be very functional for you to add someone or some role that you want to be within the portal so you can draft your own role. And then since this will be handled by it or also as the admins, they automatically add that role to the portal. So as for the portal permissions matrix, you can see that the admin portal is the manager for all the clinical operations, the governance and the access review. And then there's also the blinded case content which says denied, meaning that the admin cannot see any of the blinded cases that would have been adjudicated. So maybe if it.

Tariro Makadzange: So I get that, but you're still missing a role for the medical officer and for

Tinotenda Chibongore: Okay,

Tariro Makadzange: the monitor. So the medical officer must be able to enter the case information, but

Tinotenda Chibongore: okay,

Tariro Makadzange: not any of the adjudication reviews. They don't see that. The medical monitor sees

Tinotenda Chibongore: okay.

Tariro Makadzange: the case and the adjudication results. So if you

Tinotenda Chibongore: So.

Tariro Makadzange: could change, you know, make sure that we have those two roles.

Tinotenda Chibongore: Okay,

Tariro Makadzange: Medical

Tinotenda Chibongore: so

Tariro Makadzange: officer.

Tinotenda Chibongore: I could probably show you. But like I said, this portal is divided into four pages depending on the role that has been assigned. So I could change your role for the sake of testing so that you can look at the monitor, the medical offices portal, because it's different from the admin portal

Tariro Makadzange: But why. So why is the admin not able to see everything?

Tinotenda Chibongore: according

Tariro Makadzange: So this.

Tinotenda Chibongore: to the

Tariro Makadzange: Isn't

Tinotenda Chibongore: SOPs.

Tariro Makadzange: this the superstation where

Tinotenda Chibongore: Okay.

Tariro Makadzange: I can see all of the people assigned, or is it not set up that way?

Tinotenda Chibongore: So the admin portal was set up for user management and app integrations. So if you want your specific role so that you can see everything that's happening, we could probably branch that. But on the SOPs that I was given, the admin was only for setting up profiles as well as configuring

Tariro Makadzange: Yeah.

Tinotenda Chibongore: sites and also mapping the, the canonical fields. And also the last thing was integrating the app with other apps like outlook, like SharePoint. So it was mainly for user management and app integrations. That was the

Tariro Makadzange: Okay,

Tinotenda Chibongore: admin portals flow.

Tariro Makadzange: so

Tinotenda Chibongore: So

Tariro Makadzange: it sounds

Tinotenda Chibongore: if.

Tariro Makadzange: like you need to have an owner portal that has

Tinotenda Chibongore: Okay,

Tariro Makadzange: all of the access

Tinotenda Chibongore: okay, okay,

Tariro Makadzange: so you can have an admin

Tinotenda Chibongore: okay.

Tariro Makadzange: role and then you can put me as an owner role that has full disability. Okay, so

Tinotenda Chibongore: All right.

Tariro Makadzange: this is really around the admin component.

Tinotenda Chibongore: Oh yeah. Yes, this is just the admin component. As for the rest of the tabs you're seeing down there, it's all about them integrating. As for the studies, they're integrating with real time. The endpoints, they're integrating with SharePoint, they're integrating with teams, they're integrating with Outlook so that everything can be done in sort of an automatic way. Especially the data ingestion, they don't have. We don't have to download and upload. So the main suggestion was that they would integrate it with real time such that whenever there is new data that is entered, it is automatically uploaded to the monitors portal and then the monitor can check it for quality for the fields and then assign an adjudicator. So that was the main purpose for the, for the admin portal. So as per your suggestion, I'll have a prototype for the owner's portal that will include everything as the super user for the whole portal.

Tariro Makadzange: Okay. All right.

Tinotenda Chibongore: Okay,

Tariro Makadzange: Okay,

Tinotenda Chibongore: so for

Tariro Makadzange: so the chairperson's portal, is that the next one you want to show me?

Tinotenda Chibongore: I want to show you for the next. I want to show you the, the monitors, the monitors portal here.

Tariro Makadzange: Okay, so

Tinotenda Chibongore: So

Tariro Makadzange: how

Tinotenda Chibongore: I

Tariro Makadzange: do

Tinotenda Chibongore: would

Tariro Makadzange: I

Tinotenda Chibongore: need.

Tariro Makadzange: get to that

Tinotenda Chibongore: So

Tariro Makadzange: portal?

Tinotenda Chibongore: you, you just log out. Let me just set it up here. All right. So you can log back in now. All right, so this is now the monitors portal. As you can see they have the operational progress dashboard. This one is a bit outdated, I updated it yesterday. I'll show you the New one. So they have their studies, their sites, the adjudicators, the status of adjudications, the visit numbers. And they can also check how many adjudications were done from let's say the 7th of September to the 21st of September. So the monitor is there for quality control, quality evaluation, assigning the adjudicators to their cases and also uploading the data. So they're just looking at the adjudications that were done and when they were done, the overall completion. So it's just for, for quality control. So I would also include this for, for your super user portal. This is also what you have for you to see the progress that is being, that is being done. So that's the dashboard if you go on to the air, to the imports.

Tariro Makadzange: Just a quick question. Where

Tinotenda Chibongore: Yes.

Tariro Makadzange: is the data coming from?

Tinotenda Chibongore: Okay. So the

Tariro Makadzange: For

Tinotenda Chibongore: day.

Tariro Makadzange: the.

Tinotenda Chibongore: Yeah.

Tariro Makadzange: For. For this? Because the. Yeah, tell me where the data is coming from.

Tinotenda Chibongore: Okay. So for the testing we are using data from real time. Since we have now securely hosted on our secure servers, we are using data that is downloaded from real time, small data packets. We are just chopping it up so that we can get a small sample that we are using for adjudication. But for the actual, once you validate, you approve, everything is ready. We are going to be mapping and connecting this portal to, to real time. If you also want, we can also connect it to Crelia for quality control so that the data is automatically coming from real time or the data that's been filled before just is ingested all the way and just gets into the app. But for now we are uploading CSV.

Tariro Makadzange: How do you know the real time data is the data that is now complete? So where are you checking that? Because the EDC is

Tinotenda Chibongore: Yes.

Tariro Makadzange: where the data is marked off as complete. So help me understand the workflow and the process.

Tinotenda Chibongore: Okay. For checking that the data is complete. If you go to real time imports, there's a tab that is below the dashboard tab where it says real time imports there. Can you just click on it here? All right, so as you can see, we have those that have passed and those that have failed. So this is where it checks for the data that is complete and the data that is not complete. So if an import fails, it means that it's not complete. If you see there's one that says 100% and then it has rows processed, the participants, the visit number as well as the blinded information there. So we are using field mapping so that it can map out if the data is complete or not. However, if you wish for us to use EDC it would be better because Oracle, we are hosting on Oracle Cloud here, our portal and EDC is also part of Oracle. So it would be easier for us to connect our portal with EDC rather than getting an API from real time and then connecting that API. So if you think using Data from EDC is more cleaner. I would say yes to that. And we would start mapping out the data so that it can accept EDC data. Yeah,

Tariro Makadzange: And can you get the data from the edc because you don't really have the API from Oracle either.

Tinotenda Chibongore: I'm saying we getting the API is easier from Oracle because we are already hosting the this app on Oracle cloud. So it's easier to map out connections between our two portals rather than getting an API from real time. It's harder to get the API from real time rather than Oracle.

Tariro Makadzange: Are you sure about this?

Tinotenda Chibongore: Yeah, from what I asked. Yes. I haven't made my own inquiries. I just asked Emmanuel which one is more easier to connect. But if integration takes a long time already, we can download results from edc. We can download our results, everything, our patient data. So it's only a problem of changing the mapping of the data since the presentation of data from real time is different from edc. So it's just changing a small script so that it can be able to map out the data from etc. So I do get the issue of the data.

Tariro Makadzange: Is the real time mapping now complete? Because I'm

Tinotenda Chibongore: Yes,

Tariro Makadzange: told that this is to launch on Friday, so is the real time mapping now complete?

Tinotenda Chibongore: the real time mapping is complete by only using. By first downloading the data and then uploading it. Obviously with the help of Elliot, we can just automate that. The downloading and the uploading.

Tariro Makadzange: Okay,

Tinotenda Chibongore: Yes,

Tariro Makadzange: and then explain this. So this has 0 rows process 00. But it says

Tinotenda Chibongore: yes,

Tariro Makadzange: monitor QC

Tinotenda Chibongore: yes.

Tariro Makadzange: is required. So what's the difference between these three?

Tinotenda Chibongore: Okay. So when I was testing, I wanted to make sure that it's not just giving me fake results on uploading. So I uploaded data that was not mapped out in the way real time maps out its

data, like the values, the rows as well as the columns. So I just changed a few things within those files, a few numbers, a few rows. So automatically when it sees that, it rejects that data because it recognizes it as bad data or outer data or probably faked data.

Tariro Makadzange: Okay,

Tinotenda Chibongore: Yes.

Tariro Makadzange: so what happens after the real time imports?

Tinotenda Chibongore: Okay, so everything goes to your patient, to your patient database, as you can see there.

Tariro Makadzange: Okay, so help me understand. So you're getting this.

Tinotenda Chibongore: Okay, so for.

Tariro Makadzange: So you don't want to be sending to the adjudicator missing data. So why

Tinotenda Chibongore: Yes,

Tariro Makadzange: is this data missing?

Tinotenda Chibongore: it's not really missing if you look at it the way it maps out from real time. Go back to visit 4. So the way it maps from real time, real time as a way of duplicating a row, let's say it says create nine, it will duplicate it something like four times and then on the fourth part that's where it will have the answer to that. So you look at your visit date, it will probably come out like

Tariro Makadzange: So

Tinotenda Chibongore: four times.

Tariro Makadzange: why can't we clean that up in our import? Because if you're

Tinotenda Chibongore: For clear.

Tariro Makadzange: in adjudicator, this is going to be a bit annoying.

Tinotenda Chibongore: No, for this. This is just for the monitor. The adjudicators only get clean data.

Tariro Makadzange: Okay.

Tinotenda Chibongore: Yes, this is just for the monitor to see if the data was ingested correctly, if the rows that were supposed to be read were read correctly. So this is just for the monitor. However, if you want, I can clean it out so that it just shows the things that are required.

Tariro Makadzange: And my question is, why do we. So you're going to have a human do the qa, right? Why

Tinotenda Chibongore: For

Tariro Makadzange: can't

Tinotenda Chibongore: now

Tariro Makadzange: AI do this? Well, because the human. What is the human? Actually, just checking.

Tinotenda Chibongore: they're just checking if the data was ingested correctly like from the correct row. If you look at the, the last, the last column there it says sources, it shows where the data was taken from. So they just looking at that. But if you want AI, AI is fast actually in this thing because before, even in uploading the data we can just clean it up, look at it with AI and then within the reviews we can also integrate, integrate AI for, for the approval. Quality control approval. Yeah. So if you say you.

Tariro Makadzange: I would do that because I think the human here, I'm not sure. Oh, I meant to be on my next call. I'm not sure the human adds that much additional value.

Tinotenda Chibongore: Okay, okay. All right, so I'll take that into consideration and I'll, I'll. I'll try to integrate AI into approval of the, of the packages. So since you said we have a coconut, just quickly go to assignments.

Tariro Makadzange: So actually, let's see, just respond to this.

Tinotenda Chibongore: Okay,

Tariro Makadzange: Okay, so my 12 clock can't. So I think we're. Okay, so yes. Can we integrate AI here? Because I think the AI qc. So let's think through what are we asking the human to do and then create

Tinotenda Chibongore: okay,

Tariro Makadzange: the AI skill or agent so that it does the same thing over and over and over again.

Tinotenda Chibongore: okay,

Tariro Makadzange: You know, but,

Tinotenda Chibongore: okay

Tariro Makadzange: but to be honest, this does not really need AI, Right. So part of it is thinking, what is the task? What is the role? Does it actually need AI intelligence or you just need an algorithm that checks this versus and is that correct? So yeah,

Tinotenda Chibongore: okay. So for, for this one, we don't really need to integrate AI for, for. For such a small task because it can simply be mapped out by a script. A script

Tariro Makadzange: yeah,

Tinotenda Chibongore: can check all this and, and just validate or not validate. So yeah.

Tariro Makadzange: so I would do that

Tinotenda Chibongore: Okay,

Tariro Makadzange: because,

Tinotenda Chibongore: okay,

Tariro Makadzange: you know, the goal is to minimize the human input as much as possible on the things that can be routinely done by an algorithm or by

Tinotenda Chibongore: okay

Tariro Makadzange: artificial

Tinotenda Chibongore: okay.

Tariro Makadzange: Intelligence. Okay, then visit Reconstruction qc. What. What is this?

Tinotenda Chibongore: So you're simply going to inspect again. It's also another way of in. So once you, you inspect, you're going to also assign. So this is for single assignments, probably for just a few cases. So if you go to, after reviewing your longitude now you look first go to the drop down where you say select active adjudicator. No, just where you were. There's select active adjudicator there. Yes. And then you assign that adjudicator as the first reviewer. You can also go back to reviewer A and choose the second adjudicator. Since there will be two adjudicators for each case, you choose your reviewer B from. From. From. From the, from the drop down and then you assign your. If you are B. Yes. So

Tariro Makadzange: So again

Tinotenda Chibongore: this is

Tariro Makadzange: does

Tinotenda Chibongore: just

Tariro Makadzange: this, does a human need to do that?

Tinotenda Chibongore: like you said, this can all be done by an algorithm because we. Once you have your pool of adjudicators, it's just auto assignment. That's what we need. So for auto assignment I had. If you go to assignments there, there's a tab that says Assignments on. On your left. Yes. So there's an auto assign a B to all. Like it just auto assigns from the pool of adjudicators for every case. Since you said that we should have a script that checks and validates the packages, I think once that is all done, you can safely just auto assign every case to everyone like to every adjudicator that is that is available. So for depending on the contracts that they would have Signed we were thinking probably like 50 cases per adjudicator at a time. Yeah.

Tariro Makadzange: Okay,

Tinotenda Chibongore: Yes.

Tariro Makadzange: so.

Tinotenda Chibongore: So

Tariro Makadzange: So

Tinotenda Chibongore: that.

Tariro Makadzange: I think I. I think I would. Again, where the qc, the. The algorithm is much more reliable than a human kind

Tinotenda Chibongore: Yeah.

Tariro Makadzange: of going through and looking at 200 lines of information. That's. That's not humanly possible. So

Tinotenda Chibongore: Okay.

Tariro Makadzange: I would assign as much. Use an algorithm as much as possible. My only concern.

Tinotenda Chibongore: Okay.

Tariro Makadzange: So. So where it looks like. So what do I see? So this is the monitor, quality control, lab

Tinotenda Chibongore: Yes.

Tariro Makadzange: reference ranges. Why is there nothing there? Queries.

Tinotenda Chibongore: Okay. For the lab reference ranges we are already using the. We are already using the ones that come in embedded within real time. So probably there was a suggestion that was said. They say that they are different lab reference reference reference ranges from Lope and eoep. So you can add a new lab reference range probably for Creatinine. Instead of saying the usual lab reference rang that that is already. That we already use here in Zimbabwe. You can change it for. For that study that for. For the Lope instead of EOPE so that when it goes to the adjudicators they can quickly see the reference range as well as the actual. The actual. The actual

Tariro Makadzange: So

Tinotenda Chibongore: result from.

Tariro Makadzange: why haven't you put that in there as yet?

Tinotenda Chibongore: Okay, these ones, well right now we're using for. For eoPE. So they are already automatically within. Within the app. They are already embedded within the app. So these ones are for you to change probably to change or to add from different. Since the results from Nigeria will be coming from different labs.

Tariro Makadzange: Yes.

Tinotenda Chibongore: Yep.

Tariro Makadzange: So Tinita, why don't you then correctly set it up? So why is EOPE the default? So why don't you set it up? So depending on the case, it uses the EOPE reference range or it uses the LOPE reference range.

Tinotenda Chibongore: Okay so for, for me to quickly set it up, it's very simple. It would only require that I have the actual data from Lope. I haven't I had the chance to have my hands on data from Lope.

Tariro Makadzange: But the reference range you can get from herald. Because that should be in the lab operation manual.

Tinotenda Chibongore: Okay. Okay. Okay. So I'll simply do that. And I think also for the monitors portal, I think I've seen that I've added too many tabs that are quite not really required. So we're going to also scrape that off the lab reference ranges and then embed them in the code, not adding them manually. So we're going to embed them in the code. Since they are static, I don't think they would be changing once I do get my hands on them. So for, for a revision of the monitors portal we're going to remove the lab reference ranges and then I think we're only going to be left with the patient database and the assignments since quality control is now being will now be done by. By an algorithm so that we can minimize human input for quality control. Yeah.

Tariro Makadzange: Okay,

Tinotenda Chibongore: So are those the only, the only things that you. That you require for. For. For. For the monitors portal?

Tariro Makadzange: sure. For now. Yeah.

Tinotenda Chibongore: Yes. Okay so when I do that I'll do come back with an update. So let's now let me just quickly assign you as a chairperson so that you can look at the chairperson portal.

Tariro Makadzange: The. The only problem is. Well, I guess I need to see what the adjudicators see because the. The adjudicator

Tinotenda Chibongore: Okay

Tariro Makadzange: should see an actual report. So a human cannot read lines of creatinine. They need to

Tinotenda Chibongore: okay.

Tariro Makadzange: read a narrative story. So

Tinotenda Chibongore: Yes.

Tariro Makadzange: you know, Jane is a 50. All of these are pregnant. So 29 year old lady, G2P2 who presented with. Presented at 22 weeks pregnancy. The results of her ultrasound were blah blah, blah. Her baseline labs were blah. Visit

Tinotenda Chibongore: Okay.

Tariro Makadzange: 1 this happened. Visit 2 this happened. Visit 3 this happened. Delivery this and this happened. And then. Do you think this is preeclampsia or not?

Tinotenda Chibongore: Okay.

Tariro Makadzange: And laid out in a narrative that medically makes sense for somebody to read quickly and understand what the case is. So I need to see exactly how all of this is filtering together. All

Tinotenda Chibongore: Okay.

Tariro Makadzange: right, so now

Tinotenda Chibongore: So

Tariro Makadzange: you have your chairperson portal. Correct.

Tinotenda Chibongore: now. No now we are in the adjudicators bottle.

Tariro Makadzange: Okay,

Tinotenda Chibongore: So let me just quickly assign you a dummy case so that you can you, can, you can see what the, the monitors, see what the adjudicator says? Just a minute while I configure your profile. All right, you can, you can refresh your page, then we can continue. All right, so you could. So this is the adjudicators portal. You look at, you go to review eSource just like you do in real time. What's going on there? You go to review esos. The, the orange, the orange part there at the end. Yeah. Okay. All right, so this is what the adjudicator initially sees. They see the obstetric history, the medical conditions, the current medications that are being taken on for, for the patient history part. So this data is simply coming from real time. So how many pregnancies have they had before? Do they have a previous case of eclampsia and so on. So this is just the synthesized clinical evidence summary just telling you how many BP reading were done, how many tests were done and so on. And then you go on to the blinded form narrative. There's a narrative

Tariro Makadzange: What is

Tinotenda Chibongore: part.

Tariro Makadzange: this gentle severity and phenotype. What

Tinotenda Chibongore: So the

Tariro Makadzange: does

Tinotenda Chibongore: clinical.

Tariro Makadzange: this.

Tinotenda Chibongore: So the clinical severity and phenotype is the. If the. So remember we have where in real time they would enter if they. There is eclampsia or preeclampsia or not. There's a field for that. So if they did say suspected pe, they would go on to say severe or not severe. And then as the. For the phenotype, is it EOPE or is it lope? So that's clinical severity and phenotype.

Tariro Makadzange: But this doesn't make sense. Unclassifiable with severe features.

Tinotenda Chibongore: Okay, that's. It's. Is it in terms of unclassifiable. It's saying it was not able to map out at which number of weeks the PE was suspected or the PE was entered within real time. So that's the, the phenotype

Tariro Makadzange: Okay, but

Tinotenda Chibongore: part.

Tariro Makadzange: this doesn't make sense. So just think about it. So I'm adjudicator. I don't want to be asking questions that are complicated.

Tinotenda Chibongore: Okay.

Tariro Makadzange: I want it to be simple. So I don't understand. So I'm in the CMO here and I just don't get what this field means. So

Tinotenda Chibongore: Okay.

Tariro Makadzange: either you explain what the field means or is it useful. This is the site clinician deciding. Correct.

Tinotenda Chibongore: Yes. This is the cycling missionary.

Tariro Makadzange: Is there a reason we want to put the site clinician's opinion to be visible to the adjudicator?

Tinotenda Chibongore: I don't think there would be any reason would want the site clinicians diagnosis on there.

Tariro Makadzange: Okay, so no human can read this

Tinotenda Chibongore: Okay,

Tariro Makadzange: narrative?

Tinotenda Chibongore: so

Tariro Makadzange: That is

Tinotenda Chibongore: for

Tariro Makadzange: not

Tinotenda Chibongore: this,

Tariro Makadzange: correctly

Tinotenda Chibongore: for this part.

Tariro Makadzange: instructed.

Tinotenda Chibongore: For this part. Yeah, we had. So you see there would be a GPT model there probably GPT4O from our enterprise, from our enterprise plan where it would just crumble it up and then give you that narrative that says a 34 year old woman presents with this and this and that. So in the narrative prose it's still in testing, it's not yet integrated with AI. So that's what the AI button would be for. To change all these sections into a clean narrative that can easily be read and adjudicated for the narrative pros. Yeah.

Tariro Makadzange: Why would I need to press that as the adjudicator? Why

Tinotenda Chibongore: So

Tariro Makadzange: don't

Tinotenda Chibongore: you.

Tariro Makadzange: you have that as the workflow already in place? Because again, you want to make it so simple

Tinotenda Chibongore: Okay. All right.

Tariro Makadzange: for the adjudicator. So it literally takes 10 minutes to adjudicate and I might actually put it right at the top in bright red because that's probably the only thing they need to read.

Tinotenda Chibongore: Okay.

Tariro Makadzange: And then

Tinotenda Chibongore: So

Tariro Makadzange: if they want to look at the

Tinotenda Chibongore: yes.

Tariro Makadzange: details, they can then look at all of this but like a real medical narrative. So this should be written like an FDA submission medical narrative.

Tinotenda Chibongore: Okay.

Tariro Makadzange: So 24 year old G3P0 enrolled in such and such a condition. Exclusion. She met the following inclusion exclusion criteria.

Tinotenda Chibongore: Criteria.

Tariro Makadzange: This

Tinotenda Chibongore: Yeah.

Tariro Makadzange: is the clinical course, et cetera, et cetera. So we need to create the templates for that adjudication

Tinotenda Chibongore: Okay.

Tariro Makadzange: based on the protocol and the data that's, that's available and that should be in bright red. That's the first thing that I would go on to as the, as the. So I guess this is close up. So all of these will close up. Right. So

Tinotenda Chibongore: Yes. Yes.

Tariro Makadzange: the first thing is adjudication narrative. And that's bright

Tinotenda Chibongore: All right.

Tariro Makadzange: orange.

Tinotenda Chibongore: Okay. Yeah.

Tariro Makadzange: And that

Tinotenda Chibongore: All

Tariro Makadzange: gives

Tinotenda Chibongore: right.

Tariro Makadzange: me the summary. And then here, blood pressure. Why is there only one blood pressure?

Tinotenda Chibongore: Okay. So this, the adjudications will be done per, per visit. So if you look at the top part where it says

Tariro Makadzange: No, no,

Tinotenda Chibongore: V1,

Tariro Makadzange: no. Why is the adjudication per visit?

Tinotenda Chibongore: those were the instructions I got from, from the meetings we have been having saying that they wanted to actually see where the, the actual pe started. Is it in visit 1 and visit 2 or visit 3? So for each case it means that for one case there are six sign offs that are being done.

Tariro Makadzange: I see. Okay,

Tinotenda Chibongore: Yes. So there's visit two, visit three going on and on and on. So for these ones, this is dummy data. It's not complete. So if you

Tariro Makadzange: so

Tinotenda Chibongore: go back.

Tariro Makadzange: you also need to. So again, put yourself in the foot of the adjudicator. They're not part

Tinotenda Chibongore: Okay,

Tariro Makadzange: of your study. You don't

Tinotenda Chibongore: okay,

Tariro Makadzange: want to get them annoyed.

Tinotenda Chibongore: okay.

Tariro Makadzange: So, you know, I don't know what this unclassifiable means. I would,

Tinotenda Chibongore: All right.

Tariro Makadzange: you know what, what does this, this mean? Somebody

Tinotenda Chibongore: All right.

Tariro Makadzange: didn't have a visit 5. Right.

Tinotenda Chibongore: Yes,

Tariro Makadzange: So the adjudication only happens when you finish the study. When,

Tinotenda Chibongore: the study.

Tariro Makadzange: when

Tinotenda Chibongore: Yes.

Tariro Makadzange: do you.

Tinotenda Chibongore: Yeah,

Tariro Makadzange: Yeah. So

Tinotenda Chibongore: well,

Tariro Makadzange: you need

Tinotenda Chibongore: so.

Tariro Makadzange: to explain visit 5 did not happen. And the reason, because some people won't have visit five is the delivery visit, if I

Tinotenda Chibongore: Yes.

Tariro Makadzange: recall correctly, the eope. So

Tinotenda Chibongore: Yes.

Tariro Makadzange: why is that information not available?

Tinotenda Chibongore: All right. This is just because of the, the file was not complete. It's, it's only about the file, not the actual, the actual data not being entered since we're using dummy files.

Tariro Makadzange: So I would put visit five delivery. You know, explain to me this is delivery visit source from hospital medical records. Then the

Tinotenda Chibongore: Yes,

Tariro Makadzange: information that you have

Tinotenda Chibongore: okay.

Tariro Makadzange: and there must be a Delivery summary outcome, etc. Etc. And the correct narrative there. So you're

Tinotenda Chibongore: Okay.

Tariro Makadzange: saying

Tinotenda Chibongore: So

Tariro Makadzange: that at each visit you adjudicate each visit. Correct.

Tinotenda Chibongore: yes. So if you go back to visit one.

Tariro Makadzange: So where do I adjudicate that visit?

Tinotenda Chibongore: So go back. Yeah. Visit one. Yeah. So after looking at the blood pressure, probably the clinician wants to look at the, the lab results that we were told to

Tariro Makadzange: Yeah,

Tinotenda Chibongore: pull

Tariro Makadzange: yada

Tinotenda Chibongore: out. That's

Tariro Makadzange: yada.

Tinotenda Chibongore: the.

Tariro Makadzange: I finished

Tinotenda Chibongore: Yeah.

Tariro Makadzange: yada yada yada yada. So then

Tinotenda Chibongore: Okay.

Tariro Makadzange: where do I

Tinotenda Chibongore: So

Tariro Makadzange: put

Tinotenda Chibongore: just

Tariro Makadzange: my

Tinotenda Chibongore: so the first thing is you approve somebody and sign.

Tariro Makadzange: approved summary and signed.

Tinotenda Chibongore: Yes. So you can now go down and that this is now the actual adjudication part per visit.

Tariro Makadzange: Wait. Okay, so visit one. I've approved summary and signed. Okay,

Tinotenda Chibongore: Yes.

Tariro Makadzange: then

Tinotenda Chibongore: And you.

Tariro Makadzange: this is still visit one data.

Tinotenda Chibongore: Yes. And you are now, you are now adjudicating. So when you approve the summary and signed, you approving.

Tariro Makadzange: Wait, sorry, you're Confusing me. Just,

Tinotenda Chibongore: Yes.

Tariro Makadzange: let's just slow down here because this

Tinotenda Chibongore: Okay.

Tariro Makadzange: is not user friendly. So I've,

Tinotenda Chibongore: Okay.

Tariro Makadzange: I've looked at the labs and I've said I've looked at the labs. You're going to have a narrative for each visit. This isn't really making sense to me. You're going to have a narrative for each visit. So, so maybe before answering, just write down the comments that I'm putting through. So, so that you can hear me kef carefully.

Tinotenda Chibongore: Okay.

Tariro Makadzange: You want to make it simple. So I'm a clinician, I'm adjudicating a case. I don't just adjudicate visit 1, 2, 3, 4. I take the whole case in totality, right?

Tinotenda Chibongore: Okay.

Tariro Makadzange: 22 year old, first pregnancy. This, this, this, the ultrasound showed this, the lab showed this, this, this, this, this. Then I can say, okay, I think this person has preeclampsia or they don't have preeclampsia. And you should make it simple for me to give you my opinion.

Tinotenda Chibongore: Okay.

Tariro Makadzange: It should be simple where I put in this is my opinion. Then when

Tinotenda Chibongore: Okay.

Tariro Makadzange: I finish, you can say to me, if your question is when was the onset of preeclampsia detected,

Tinotenda Chibongore: Yes.

Tariro Makadzange: then have a space where it says visit one, Preeclampsia, yes, no, and whatever the classification, visit two, yes, no classification, visit three, yes, no classification, et cetera. So

Tinotenda Chibongore: Okay.

Tariro Makadzange: that is totally intuitive. Completely

Tinotenda Chibongore: All right.

Tariro Makadzange: intuitive and straightforward. I should not have to email anyone, you know, because for me, immediately and feeling anxiety, I'm like, what, what the hell is unclassifiable, right?

Tinotenda Chibongore: Okay. Yeah.

Tariro Makadzange: And then I have a blood pressure recheck interval. Interval said zero minutes, is that correct?

Tinotenda Chibongore: Yes. The interval should be between five minutes to, to 12 hours.

Tariro Makadzange: So

Tinotenda Chibongore: So.

Tariro Makadzange: it should be documented what that interval is. Because if you say to me zero minutes, then you know, I, you know, I mean, I guess it has calculated and this is the same

Tinotenda Chibongore: Yes.

Tariro Makadzange: time,

Tinotenda Chibongore: Yeah, it just

Tariro Makadzange: but

Tinotenda Chibongore: calculated

Tariro Makadzange: hopefully.

Tinotenda Chibongore: the time thing.

Tariro Makadzange: And then the biochemistry. So nobody reads labs this way. I've never read labs this way.

Tinotenda Chibongore: Okay.

Tariro Makadzange: So. And these are the only labs that, that we have. Platelets, creatinine.

Tinotenda Chibongore: That, no, that I needed for, for the adjudication portal, there was an SOP that, that says we only need the platelets, the Creator 9 as LDH as well as the protein result.

Tariro Makadzange: But why? Because I'm a clinician. Are they anemic?

Tinotenda Chibongore: Yes. Yeah.

Tariro Makadzange: Is there bicarbonate? Correct.

Tinotenda Chibongore: Okay.

Tariro Makadzange: So

Tinotenda Chibongore: So.

Tariro Makadzange: why. You know, obviously I need these, these are part of the criteria for preeclampsia. So maybe you can highlight the platelets, the creatinine, et cetera. But if I have, I don't know if you have a bun, you know, if you have a full renal panel or you have a full CBC or any other parameters. If this is all you have, you know, that's fine. Personally, this is hard to read because now this is across the entire screen. Why not put

Tinotenda Chibongore: Okay.

Tariro Makadzange: it as hematology, hemoglobin,

Tinotenda Chibongore: All right.

Tariro Makadzange: hematopoids, platelets, et cetera,

Tinotenda Chibongore: Okay.

Tariro Makadzange: renal function, creatinine, urinalysis. All of the things in the urinalysis, LFTs, what you have in the LFTs, and the LDH, urine protein, creatinine ratio. Did you calculate it? And it's missing in this one, but it's there in others. If it's missing because for that patient, for whatever reason it was unavailable, you should say unavailable because I'm

Tinotenda Chibongore: Okay.

Tariro Makadzange: immediately thinking, oh my God, these people didn't do this study correctly. There's so much missing data.

Tinotenda Chibongore: Okay. All right.

Tariro Makadzange: So making it very clear, this is the data that was collected, this is the data available. Because you want the clinician to make a decision with the totality of the information with the

Tinotenda Chibongore: Okay.

Tariro Makadzange: ultrasound scan report.

Tinotenda Chibongore: All right.

Tariro Makadzange: I don't see an ultrasound scan. The scan report, because

Tinotenda Chibongore: Okay.

Tariro Makadzange: if it was me, I would want to read the ultrasound scan report.

Tinotenda Chibongore: Okay.

Tariro Makadzange: Because

Tinotenda Chibongore: So

Tariro Makadzange: you're asking me to make a decision based on all of the information that I have.

Tinotenda Chibongore: all right. I think for the adjudication part there's a complete reconstruction that's required because from all these meetings, that's why I sent a message so that you could validate these things. The ones that I presented to were saying it's okay for them. So I was doing based on their suggestions. But since you now have a different narrative and I actually know what is required because like you said, I don't think for someone who is not really, who doesn't need to know much about the study, there is no need to have a portal that is this complicated. So for my suggestion from what you have told me

Tariro Makadzange: Okay,

Tinotenda Chibongore: the

Tariro Makadzange: so

Tinotenda Chibongore: first thing.

Tariro Makadzange: it's not the complication. So here we're thinking like a clinician. So you're

Tinotenda Chibongore: Yes,

Tariro Makadzange: me, Harvard trained doctor. Now pretend I was an obstetrician, which I'm not. You know when I look at and see a patient and you're asking me to make an opinion, you refer an ID patient to me. I need to know all of the data.

Tinotenda Chibongore: Yes,

Tariro Makadzange: You

Tinotenda Chibongore: that's

Tariro Makadzange: can't

Tinotenda Chibongore: true.

Tariro Makadzange: just cherry pick because this patient could be anemic and then they die and then you want me to classify. I also want to know, in the obstetric nodes that you have, did they have excessive bleeding when they deliver?

Tinotenda Chibongore: Okay.

Tariro Makadzange: Might not be something that's in your edc, but in your real time or wherever it is. I need to know because if that person then died, I need to understand why they died. Then you want to minimize the amount of work that comes back to you. You

Tinotenda Chibongore: Okay.

Tariro Makadzange: want to try and create almost a human free system. Because the

Tinotenda Chibongore: Okay.

Tariro Makadzange: next thing I'm going to ask you, because AI could adjudicate all of these cases, you don't need human reviewers.

Tinotenda Chibongore: Yes.

Tariro Makadzange: What I want you to also do is create a back portal where AI concurrently adjudicates. Because if you have the icog, right, that's what they call them, acog, American College of Obstetrics and Gynecology, the WHO guidelines, you know, if Zim, Tanzania, Rwanda, blah blah blah, have the Edlers or whatever guidelines that could be the knowledge base of, of the AI and PubMed publications, for example, and the AI is probably going to be much better than all the human adjudicators you have. Right.

Tinotenda Chibongore: Yeah.

Tariro Makadzange: But what we need to do now is to train the model. So at the end of this, we'll have a Data set of 2000 or whatever between Lope and Eope compared to the AI adjudicator. And you

Tinotenda Chibongore: Okay.

Tariro Makadzange: could even write a manuscript at the end of that about AI adjudication versus human adjudication

Tinotenda Chibongore: Okay.

Tariro Makadzange: and build a model for the next preeclampsia type, type of study.

Tinotenda Chibongore: Okay. Yeah. So I think I now get the point from your view because from where we were doing our sessions,

Tariro Makadzange: Onset classification, early onset, late onset. How do I know the definition? You know, I'm

Tinotenda Chibongore: It's.

Tariro Makadzange: an old. You know, most of your professors might have not read the ACOG guidelines in a long

Tinotenda Chibongore: Okay,

Tariro Makadzange: time. So

Tinotenda Chibongore: all right,

Tariro Makadzange: how am I defining it as early? So I would put early onset, whatever the ACOG definition, I

Tinotenda Chibongore: all

Tariro Makadzange: don't

Tinotenda Chibongore: right.

Tariro Makadzange: know what it is, less than 32 weeks,

Tinotenda Chibongore: Less than 32 weeks

Tariro Makadzange: whatever

Tinotenda Chibongore: here.

Tariro Makadzange: it is, then late onset, whatever that is. Postpartum onset, not here. So make it very simple and specific

Tinotenda Chibongore: All right.

Tariro Makadzange: because these

Tinotenda Chibongore: So

Tariro Makadzange: guys are university professors. They're running around, you know, just think of the people who are teaching you. They're running around private practice trying to make extra money. They want

Tinotenda Chibongore: yes.

Tariro Makadzange: to do research. This is not really paying them that much. It's really trying to, you know, get their professional opinion. They actually do it late at night. They put their kids to bed, they want to knock this off. They're going to keep forgetting until you give them a deadline. And they'll have 100 of them to do all at once. So make this super simple for them to do. And the narrative, kind of bright red here narrative. Maybe they click on this and there's a word document that describes like your most perfect, I don't know, Harvard med school or first year resident narrative in

Tinotenda Chibongore: Okay.

Tariro Makadzange: the style of an FDA type of submission. So that when you're done, anybody on the planet can look at it and say, okay, yes, this was preeclampsia. Because eventually, you know, you publish your results and you say, Roche is crap. Roche might say, okay, we want an independent third party to evaluate this.

Tinotenda Chibongore: Okay.

Tariro Makadzange: And you just say, here's the portal and read it for yourself. And you can get AI to write that narrative. Once you put the data in order, that's where you might need intelligence to write the draft narrative. Then have your human medical monitor review it. But personally, what's going to end up happen in reality is your human medical monitor is overwhelmed with all the other things that they do that they don't get round to reading it all and reading it all on time. I would make sure that I generate a Standardized narrative framework. Send it to me to approve. Generate a standardized narrative framework test test on 20, 30 actual cases. Then have it generate all, all of the, all of the narratives for the cases for adjudication and then assign internally random quality checks. You need your medical monitor, et cetera, to do random quality checks to make sure that the AI is not going rogue and is generating useful reports. But

Tinotenda Chibongore: Okay.

Tariro Makadzange: to be honest, your medical monitor should be able to do this. There's only going to be about a thousand or so of those narratives. But I would

Tinotenda Chibongore: Now it has.

Tariro Makadzange: train the AI to generate so that your human is doing very, very little. There's very few edits that the human, if any, needs to do.

Tinotenda Chibongore: Okay, so for, for, for the history part, since you said you want a complete first year residency type history, I. I could use the guideline that we have the John to be a guideline probably.

Tariro Makadzange: No, no, no, no, no, no, no, no. This has to be an FDA drafted template for

Tinotenda Chibongore: Okay.

Tariro Makadzange: the

Tinotenda Chibongore: Okay.

Tariro Makadzange: FDA

Tinotenda Chibongore: Let me just.

Tariro Makadzange: submission narrative

Tinotenda Chibongore: Okay, let

Tariro Makadzange: for

Tinotenda Chibongore: me just

Tariro Makadzange: the

Tinotenda Chibongore: look

Tariro Makadzange: fee

Tinotenda Chibongore: for it.

Tariro Makadzange: of

Tinotenda Chibongore: So,

Tariro Makadzange: seal studies.

Tinotenda Chibongore: so it's. Is the FDA approved narrative? Okay,

Tariro Makadzange: Okay,

Tinotenda Chibongore: all right.

Tariro Makadzange: all

Tinotenda Chibongore: So

Tariro Makadzange: right. So it

Tinotenda Chibongore: okay.

Tariro Makadzange: can do that while we continue. So reformat this into a structure, a board, visibility. That's a lot easier for somebody to read and understand.

Tinotenda Chibongore: Okay.

Tariro Makadzange: Yeah, and I mean personally as a doctor, this would be meaningless to me. I can't. This is not how I think. You know, it's nice

Tinotenda Chibongore: All right.

Tariro Makadzange: that you look slow, normal, blah, blah, blah. That's great. But I would want to see the full hematology and you could put hematology, Renal chemistries, renal LFTs, urinalysis imaging, if there's any imaging. So

Tinotenda Chibongore: Okay.

Tariro Makadzange: the ultrasound scan, etc, if there was any other imaging in between. I don't know if you have anyone diagnosed with TB or whatever, whatever else may have come up. So other visitors, evidence, symptoms, medications, diagnostics, whatever, whatever it is and just make

Tinotenda Chibongore: All

Tariro Makadzange: it

Tinotenda Chibongore: right.

Tariro Makadzange: easy to read so I don't have to click too much. I can make it easy to read. Then cumulative

Tinotenda Chibongore: All right.

Tariro Makadzange: interpretation, what is this then? Longitudinal per visit evidence. Okay, this is helpful. Okay, this is good. Fantastic. This is great. This makes sense to my brain. I can see okay, BP, BP is rising, coming down, etc. Not everybody is going to have a visit for it depends on when they delivered and so on. So you

Tinotenda Chibongore: Yes.

Tariro Makadzange: might color the post delivery visits a different background color

Tinotenda Chibongore: Okay

Tariro Makadzange: so that my brain immediately

Tinotenda Chibongore: okay.

Tariro Makadzange: can see, oh, the baby is born. Now who does the visit classification?

Tinotenda Chibongore: It's the, the site clinician when they, when they evaluate the date.

Tariro Makadzange: Okay, all right, so

Tinotenda Chibongore: Yeah.

Tariro Makadzange: that's fine. So maybe put site clinician visit classification or something.

Tinotenda Chibongore: Okay. Okay.

Tariro Makadzange: Final adjudication control. So let's see what's cumulative interpretation through this visit. What does this mean?

Tinotenda Chibongore: Okay. So it's the, the general site clinicians diagnosis as a whole. So it's. So across all the visits or across the visit, they did not find any support for PE and then they did not qualify the certain. It is a probable. Is it definite? The evidence completeness is 100 because they had all the data and then they did

Tariro Makadzange: So

Tinotenda Chibongore: not.

Tariro Makadzange: maybe explain, explain this because you know, cumulative through this visit by site clinician.

Tinotenda Chibongore: Okay.

Tariro Makadzange: And then explain what it is so that I'm not going to start emailing because again, remember I might be doing this at 10, 11 o'clock at night, I put the

Tinotenda Chibongore: Okay.

Tariro Makadzange: kids to sleep. I'm trying to do this very quickly, so just make it clear. Any question I would ask is. Is already. Is already answered.

Tinotenda Chibongore: Answer. Okay,

Tariro Makadzange: And I don't. What is this form? What is this?

Tinotenda Chibongore: so this is what, where the history is supp is going to be the,

Tariro Makadzange: Oh, I might,

Tinotenda Chibongore: the

Tariro Makadzange: I

Tinotenda Chibongore: narrative.

Tariro Makadzange: would sort that out because I would, I would write the narrative like I've said to you and put

Tinotenda Chibongore: Okay.

Tariro Makadzange: it bright red here because that's pretty much all I'm going to read.

Tinotenda Chibongore: Okay.

Tariro Makadzange: And then I can scroll down here for extra information as questions come up.

Tinotenda Chibongore: All right. Okay. So yeah, I, I'm getting the

Tariro Makadzange: This

Tinotenda Chibongore: picture.

Tariro Makadzange: is something you would click on and either, you know, yeah, you would click on, you'd read the narrative and the easiest way for visibility say you click on and this half is the narrative with the ability to expand it to read more easily or contracted and then these drop down menus available.

Tinotenda Chibongore: Okay, that, that, that makes sense. Okay. So I'm going to reconstruct the whole

Tariro Makadzange: And then

Tinotenda Chibongore: adjudication

Tariro Makadzange: I don't understand.

Tinotenda Chibongore: part.

Tariro Makadzange: So again, just think what does it mean for me as the reader? Why is the available? What does this mean? Why do I need that? Why

Tinotenda Chibongore: Okay.

Tariro Makadzange: not just have yes in, you know, I mean

Tinotenda Chibongore: Or

Tariro Makadzange: proteinuria

Tinotenda Chibongore: to remove it completely.

Tariro Makadzange: is

Tinotenda Chibongore: Yeah.

Tariro Makadzange: not. Proteinuria is not. Yes, no, isn't it one plus two plus three plus.

Tinotenda Chibongore: Oh, that's just the question if the proteinuria was done or not.

Tariro Makadzange: But for me

Tinotenda Chibongore: The.

Tariro Makadzange: as the adjudicator I don't care, right? So what

Tinotenda Chibongore: Yeah,

Tariro Makadzange: I

Tinotenda Chibongore: you do.

Tariro Makadzange: need is proteinuria. One plus two plus three plus not done at each of the visits. So I can immediately see, you know, it's going up, it's decreasing, it isn't decreasing urine, protein, creatinine ratio, whatever that is. Symptoms none. You know, I don't know what available, available, available means

Tinotenda Chibongore: Okay.

Tariro Makadzange: Symptoms, headache, blah, blah, blah, blah, blah. No symptoms,

Tinotenda Chibongore: Okay.

Tariro Makadzange: swollen, pedal edema or whatever. So I can see it very quickly. Medications, Tylenol, nifedipine, etc, I don't care if it's available or not. Not available. Fetal

Tinotenda Chibongore: I

Tariro Makadzange: assessment,

Tinotenda Chibongore: guess.

Tariro Makadzange: normal, normal, normal. Visit classification, you know, what's, what is this?

Tinotenda Chibongore: All right.

Tariro Makadzange: And I think for that you can just leave this screen but have the detail in the narrative and as I said by, you know, the full biochemistry, the full hematology, etc. This is just your longitudinal comparison summary or something

Tinotenda Chibongore: Okay.

Tariro Makadzange: so that I can just quickly look at this. And that way even if you have a lazy adjudicator, they at least look at this. And then after looking at this I would put this at the top because this is afterwards is where you want them to put this adjudication information.

Tinotenda Chibongore: Yes. Yeah.

Tariro Makadzange: And then I would do it as the full narrative because you're not going to say preeclampsia, yes or no after seeing the full narrative. And then if the final adjudication is preeclampsia or one of these things. Then you want to drop down visit whammy, was there preeclampsia? Visit 2, was there preeclampsia? 3,

Tinotenda Chibongore: Okay.

Tariro Makadzange: 4, et cetera. But if the whole narrative says no preeclampsia, there's really no point in going through 1, 2, 3, 4, 5. Right.

Tinotenda Chibongore: Okay. All right. So I'm going to do a complete reconstruction of the adjudicators, make it simpler, provide the narrative and add the missing lab results, add all the bloods, the biochemistry and as well as the urinalysis results also so that like you said, we can make it easier and faster for them to adjudicate. Yeah.

Tariro Makadzange: And then the differential diagnosis piece as well. We. You kind of want to make it simple for them, otherwise they won't do it, you

Tinotenda Chibongore: Okay.

Tariro Makadzange: know, so think of the sort of normal delivery other and then give them the text to put their or actually differential diagnosis alternative explanation and you can write record important alternatives considered or why non explained? If yeah, why non explained? Make this required. So make the fields required. Otherwise you're going to have missing information before they can sign and lock the adjudication report.

Tinotenda Chibongore: Okay. That's

Tariro Makadzange: And then you probably want to drop down menu of the inclusion exclusion criteria

Tinotenda Chibongore: okay.

Tariro Makadzange: so that I can very quickly see what criteria they met for enrollment.

Tinotenda Chibongore: All right.

Tariro Makadzange: Then it's clear, simple, easy for me to kind of understand what the case is and then it will literally take me 10 minutes per case

Tinotenda Chibongore: All right. So yeah, I think we're going to make it as simple as. As possible for easy

Tariro Makadzange: and then

Tinotenda Chibongore: adjudication.

Tariro Makadzange: I would avoid. So you said east, why don't you make it simple so you know, minimize locked et okay. So it becomes okay, that's my queue. All right. Okay. So this E source evidence, do you need this there? Because I have to click here for the E source action, right?

Tinotenda Chibongore: Yeah, I can just scrape that off. It's a.

Tariro Makadzange: Yeah, so that's just minimize the confusion. You know, when you build stuff, you know, many people may have PhDs, think they're smart, etc. But in

Tinotenda Chibongore: Yeah,

Tariro Makadzange: life when you're very busy, things should be fifth grade level. So just think could a grade. If I showed somebody in grade five this, will they get confused or they'll know what to do?

Tinotenda Chibongore: all right. So okay, I do get that. So you're telling me to build from the user's point of view. Yeah.

Tariro Makadzange: 100%. 100.

Tinotenda Chibongore: Yes.

Tariro Makadzange: Because you want them to complete this and you want them to give you their best expertise and you want them to be excited to do it. So I've laid down the kids kids to bed. But these guys at acrn, these are the most interesting cases that I've

Tinotenda Chibongore: Okay.

Tariro Makadzange: read in a while. I wish my obstetric residents wrote notes like this.

Tinotenda Chibongore: Okay. All right. Okay. Yeah, I do understand. So.

Tariro Makadzange: You might even put in the reference portal a link to the protocol, a link to the ACOG guidelines or whatever guidelines we quoted in in the protocol or have a tab here that says references so that it's very easy for me because I might be a professor at UZ and I haven't read ACOG guidelines in a really long time.

Tinotenda Chibongore: Okay. So yeah,

Tariro Makadzange: And you just have that portal, that's that link and you know, it's easy, easy for me to go. I don't know what DV26 score is. Would they know what it is?

Tinotenda Chibongore: They, they wouldn't. It's just completeness of data because since we are missing the imaging, we don't have imaging. So our DV deliverable.

Tariro Makadzange: Why don't you have imaging?

Tinotenda Chibongore: Usually when people only get a scan from visit One, most of them for visit two, three, and four, they don't really have any imaging information.

Tariro Makadzange: Yeah, because

Tinotenda Chibongore: They

Tariro Makadzange: they

Tinotenda Chibongore: aren't.

Tariro Makadzange: don't need imaging. But

Tinotenda Chibongore: Yeah.

Tariro Makadzange: you should put the link to the ultrasound on visit one because as an obstetrician

Tinotenda Chibongore: Okay.

Tariro Makadzange: I'd want to know. I want

Tinotenda Chibongore: All

Tariro Makadzange: to

Tinotenda Chibongore: right.

Tariro Makadzange: know how the gestational age was determined. I

Tinotenda Chibongore: Okay.

Tariro Makadzange: want to make a comment on. On that. And then I would take this out because, you know, for me as the user, I'm just going to get very confused by this. And then what's derived phenotype?

Tinotenda Chibongore: Okay. So the derived phenotype, that's the one we're talking about where we say, is it EOP or lope or postpartum? And then is it severe or not severe?

Tariro Makadzange: So I would take that out because when I log in, I'm the adjudicator.

Tinotenda Chibongore: Okay.

Tariro Makadzange: This is what I need to decide. Right? You're asking me to give you a classification. I don't need to see this. A GA AT event, I don't need. So this can come out. The derived phenotype can come out. The DV26 score can come out. You know, if you want, you could save the study. Is it the LOPE study or the EOPE study? But

Tinotenda Chibongore: Okay.

Tariro Makadzange: all this other stuff could come out. The status. You can put that there. It's not. They all must be enrolled by definition. Right.

Tinotenda Chibongore: Yes.

Tariro Makadzange: And they all should have completed the study visits. All the ones who are being pushed into adjudication would have completed their study visits, right?

Tinotenda Chibongore: Yes. Yeah.

Tariro Makadzange: Yes. So simplify this. The case number you can put. Yeah. And then if this is meaningful, and then, yeah, keep this very simple.

Tinotenda Chibongore: All right, so is there anything else? Because right now

Tariro Makadzange: Do

Tinotenda Chibongore: I can

Tariro Makadzange: I

Tinotenda Chibongore: see that.

Tariro Makadzange: need the E source evidence? I probably don't. Yeah, we talked about this. I would take that out and then I guess when I sign it will say locked, QC portal and gates. So what's here?

Tinotenda Chibongore: So this one is not yet functional, but this is where we wanted to put the links. Like you're saying, some of the important links they might need to review before they. Before they

Tariro Makadzange: IA

Tinotenda Chibongore: adjudicate.

Tariro Makadzange: master specification. So obviously this will not be there, but you

Tinotenda Chibongore: Okay,

Tariro Makadzange: might have a work instruction on how to move through the portal so that,

Tinotenda Chibongore: okay,

Tariro Makadzange: you know, nobody's emailing you. I can't find this, I can't find that. So

Tinotenda Chibongore: okay,

Tariro Makadzange: work instruction that have split

Tinotenda Chibongore: okay,

Tariro Makadzange: pictures and images and again keeps it fifth grade level on where. Where I find things, how I navigate the portal. So you could put that. And then I would put a reference literature.

Tinotenda Chibongore: okay,

Tariro Makadzange: So I would put acog,

Tinotenda Chibongore: okay.

Tariro Makadzange: I would put, you know, Zim, you know, all the countries, Zim, Rwanda, blah, blah, blah. Because I might see that a patient got drug X and I'm wondering why they got drug X.

Tinotenda Chibongore: Okay.

Tariro Makadzange: And so you can put the, you know, if there's an Edlers for Tanzania, Edlers for Rwanda whatever, the WHO guidelines for preeclampsia, the ACOG guidelines, etc. So just put the reference documents as well, so it's easy for folks to find.

Tinotenda Chibongore: Okay. All right. Would you like to look at the chairperson spot out next?

Tariro Makadzange: Sure. I think we've spent quite a bit of time here and I have a poll at one

Tinotenda Chibongore: Okay.

Tariro Makadzange: o'. Clock. So I've given you quite a lot to work on.

Tinotenda Chibongore: Yes,

Tariro Makadzange: So if

Tinotenda Chibongore: that's

Tariro Makadzange: you could

Tinotenda Chibongore: true.

Tariro Makadzange: work on that, your note taker should be there, the FDA should.

Tinotenda Chibongore: Yes.

Tariro Makadzange: The note.

Tinotenda Chibongore: Yeah. Minority. Okay, so. Okay, so thank you, doc. Thank you for, for make. For giving me the corrections that I, That I need to. To. To. To put in. I'll give you an update on Friday when I do scrape off most of the things that I. That you said I should and, and make it simpler for the

adjudicators as well, creating a super raw for you as the owner. So, yeah, I'll have an update on Friday for the prototypes for the new prototypes. Once you approve them, then I'll deploy those.

Tariro Makadzange: Okay,

Tinotenda Chibongore: Okay, thank you

Tariro Makadzange: yeah.

Tinotenda Chibongore: so much.

Tariro Makadzange: So you'll need to give me the links and chase me on it. We need to get this right. And then as

Tinotenda Chibongore: Okay.

Tariro Makadzange: I said, I would like so always kind of thinking about what can an algorithm or what can code do. This is how I think of it. What should a

Tinotenda Chibongore: Okay.

Tariro Makadzange: human do? And just

Tinotenda Chibongore: All right.

Tariro Makadzange: think of the humans in our teams. They're super busy, they have minimal skill. I mean, this is why people are telling you this is okay. And meanwhile, it's far from okay. A lot of people are learning on the

Tinotenda Chibongore: Yeah.

Tariro Makadzange: job. Learning.

Tinotenda Chibongore: Yes.

Tariro Makadzange: So what's, what do the humans really need to do? What can a code do? Can an algorithm code streamline this and do it better than a human can? And then what

Tinotenda Chibongore: Okay.

Tariro Makadzange: do you need intelligence to do? You need intelligence to act like a medical monitor. This is an intelligent thing, right. You can see the screen. It's thinking about, you know, FDA

Tinotenda Chibongore: Yes.

Tariro Makadzange: templates, etc. So I'm going to look at it, I'm going to edit it, etc. Send it to you. So you need an obstetrician, somebody

Tinotenda Chibongore: Okay.

Tariro Makadzange: who spent 20, 30 years thinking about preeclampsia. AI can do that for you. So things like your narratives, AI could do. And as I said, I think the adjudication, I think AI can probably do the adjudication better than the human. So be interesting where there's discrepancies, what the AI says versus the chairperson. And you can, I mean, this is this. You could even write up as a project, as a paper, at the end of it,

Tinotenda Chibongore: Okay.

Tariro Makadzange: your AI

Tinotenda Chibongore: Yes.

Tariro Makadzange: adjudicator, your two, I think that says two humans. And then the chairperson calls a meeting or something, or the chair third decides if there's still a problem or something. You call a meeting. I can't remember what the workflow is.

Tinotenda Chibongore: That. That's

Tariro Makadzange: Those meetings.

Tinotenda Chibongore: the workflow. Yeah.

Tariro Makadzange: Yeah, you put in your AI notetaker. And then you'll see at the end of these 2000, 1000 EOP, E1000 Lope, who is the better adjudicator.

Tinotenda Chibongore: Okay. Yeah, I do get that. So I'll try as much to have prototypes ready so that you can just validate them. But as for revamping the whole thing and deploying, I would say probably four days. So that will be next week, Monday. Yeah.

Tariro Makadzange: Okay.

Tinotenda Chibongore: Yes. Okay. Thank you, Doug.

Tariro Makadzange: All

Tinotenda Chibongore: Is

Tariro Makadzange: right.

Tinotenda Chibongore: there anything else? Yeah.

Tariro Makadzange: No, I think, I think that's, that's good. Thanks. And yeah, this is coming together very nicely. Thanks.

Tinotenda Chibongore: Okay. Thank you, doc. Have a good day.

Tariro Makadzange: Same to you. Bye.

Tinotenda Chibongore: Bye.

Chat History

Tinotenda Chibongore: <https://adjudication.acrncloud.com/>